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IMPLEMENTING OVERDOSE PREVENTION AND RECOVERY CENTERS TO COMBAT WEST VIRGINIA'S OPIOID CRISIS

West Virginia is confronting a profound substance use disorder (SUD) crisis, particularly with opioids, that has devastated its population's health, economy, and social cohesion. A survey by West Virginia University estimates that 359,880 individuals engaged in drug use within the past year, an alarming figure for a state with just under 1.8 million residents. Nearly 1 in 5 West Virginians are directly affected, placing the state at the epicenter of America's addiction crisis (Adams, 2024). Overdose death rates surged from 28.2 per 100,000 in 2010 to 80.9 per 100,000 by 2022—a 135% increase—now 85.6% higher than the national average (Rice, 2024).

The economic toll is staggering. Research estimates the crisis costs West Virginia \$11.3 billion annually, equal to 12% of the state's GDP, factoring in healthcare costs, treatment expenses, criminal justice burdens, and lost productivity (Rice, 2024). Children bear heavy consequences as well: 83% of child welfare removals involve homes with known drug use, perpetuating intergenerational harm (Samples, 2024). Addressing this crisis demands innovative, evidence-based solutions.

West Virginia's current approach includes prevention, treatment, and harm reduction policies. Medicaid is central to these efforts, funding treatment for around 50,000 residents annually. Programs like the 2018 SUD Waiver support methadone treatment and residential care, with projected spending reaching \$140 million by 2025, alongside \$70 million for medication-assisted treatment (MAT) (Rice, 2024). Harm reduction initiatives include naloxone distribution—a life-saving measure—with a record 97,780 kits distributed in 2023

(Government of West Virginia, 2024). While these measures save lives, they remain reactive and fail to address the underlying causes of opioid misuse. Educational campaigns have been introduced to discourage substance misuse, particularly among youth, but require years of sustained effort to achieve tangible results. Despite these actions, the crisis persists, highlighting the need for more effective interventions.

To tackle this epidemic, West Virginia should implement Overdose Prevention and Recovery Centers (OPRCs) modeled on the success of OnPoint NYC in New York City. These centers provide medically supervised spaces where individuals can use substances safely while accessing essential healthcare, harm reduction services, and recovery support. The proposed policy will establish five OPRCs in the hardest-hit counties: Kanawha County (Charleston), Cabell County (Huntington), Monongalia County (Morgantown), Raleigh County (Beckley), and Wood County (Parkersburg). These locations reflect high overdose rates, vulnerable populations, and limited access to treatment, ensuring the greatest impact across urban and semi-rural communities.

Each OPRC will deliver comprehensive services. Overdose prevention will be the primary focus, with trained medical staff prepared to intervene immediately and prevent fatalities. The centers will provide clinical care, including wound treatment, HIV and Hepatitis C testing, and buprenorphine prescriptions to support recovery. Drop-in centers will offer hot meals, hygiene facilities, and digital connectivity, restoring dignity and building trust with participants.

On-site drug testing will help identify dangerous contaminants like fentanyl and xylazine, reducing accidental overdoses. Mental health services—free therapy, crisis intervention, and psychiatric care—will address co-occurring disorders that frequently accompany substance misuse. Holistic services such as acupuncture and meditation will complement medical care,

offering pathways to emotional and physical well-being. Outreach teams will engage with neighborhoods to provide harm reduction education, clean hazardous waste, and address public safety concerns. Finally, professional development programs will offer job training and resume-building workshops, promoting reintegration into society and economic stability.

The estimated cost of this policy is \$20 million annually, funded primarily through State Opioid Settlement Funds. Additional support will come from federal grants via SAMHSA and Medicaid's SUD Waiver program, while partnerships with private organizations and harm reduction non-profits will supplement outreach and support initiatives. In the long term, the policy will generate significant cost savings by reducing emergency healthcare expenditures, criminal justice costs, and burdens on first responders.

The success of OPRCs is well-documented. Since its launch in 2021, OnPoint NYC has reversed over 1,000 overdoses without a single fatality while connecting individuals to recovery services and improving neighborhood safety (Onpoint NYC, 2024). Geneva's Quai 9, operating for over 20 years, has demonstrated long-term success in reducing infectious disease transmission, minimizing public drug use, and fostering reintegration into social services. These examples provide strong evidence for the effectiveness of OPRCs as harm reduction tools.

Critics argue that such centers encourage drug use or face community resistance. However, evidence from OnPoint NYC and Quai 9 refutes these claims. OPRCs reduce public drug use, leading to cleaner neighborhoods and fewer discarded syringes, while serving as gateways to recovery by connecting participants with counseling, treatment, and rehabilitation services. Although upfront costs may raise concerns, the long-term financial and societal benefits far outweigh initial investments. To address resistance, the policy will incorporate

public education campaigns and continuous performance monitoring, ensuring transparency and accountability while fostering community trust.

West Virginia's opioid epidemic demands bold, transformative action. Current approaches have fallen short of reversing the devastating trends. Implementing Overdose Prevention and Recovery Centers, modeled on proven successes like OnPoint NYC and Quai 9, offers a life-saving, evidence-based response. These centers will reduce overdose deaths, improve public health, and provide critical pathways to recovery, while creating safer, healthier communities. West Virginia has the opportunity to lead in harm reduction and set a new standard for addressing the opioid crisis. The time to act is now to save lives and rebuild communities for a stronger, healthier future.

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